

Major interactions (continued)

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Triazolam + Temazepam

additive CNS depression

Additive sedation, psychomotor impairment and next-day residual effect. The mechanisms differ (GABA-A, H1, orexin, MT1/MT2) so the effect is not detected by any duplication check that reasons from drug class alone.

Triazolam + Temazepam

duplication

Same class, same receptor site, same indication. Also recorded as additive CNS depression above; both entries are intentional, because a duplication check and a sedation check should each catch this.

CYP3A4 INHIBITION

Daridorexant + Cimetidine

metabolism

89% of daridorexant's metabolic clearance is CYP3A4, so its exposure is close to a direct function of 3A4 activity. Next-morning driving impairment is the dose-related risk the label caps for.

Loperamide + Cimetidine

metabolism

Loperamide's safety rests on almost none of it reaching the systemic circulation. Cimetidine inhibits CYP3A4 — one of its two clearance routes — raising systemic exposure toward the range where hERG blockade prolongs QT. The FDA boxed warning for loperamide is about exactly this scenario.

Triazolam + Cimetidine

metabolism

Triazolam is cleared almost entirely by CYP3A4 and has a sub-milligram therapeutic dose, so it has essentially no margin. Cimetidine inhibits CYP3A4. Expect prolonged sedation, next-day impairment and anterograde amnesia.

SYMPATHOMIMETIC AND UNOPPOSED ALPHA

Pseudoephedrine + Amphetamine IR

synergism

Additive sympathomimetic effect. Amphetamine and pseudoephedrine share the indirect noradrenaline-releasing mechanism; caffeine adds through adenosine antagonism. None of these is detected by a same-class duplication check.

Pseudoephedrine + Amphetamine XR

synergism

Additive sympathomimetic effect. Amphetamine and pseudoephedrine share the indirect noradrenaline-releasing mechanism; caffeine adds through adenosine antagonism. None of these is detected by a same-class duplication check.

Pseudoephedrine + Propranolol

antagonism

The textbook unopposed-alpha interaction: pseudoephedrine drives alpha-1 vasoconstriction while propranolol blocks the beta-2 vasodilation that would normally offset it. Hypertensive episodes with reflex bradycardia are documented, including in people whose blood pressure is otherwise well controlled.

ABSORPTION

Propranolol + Colestipol

absorption

Specifically documented on the colestipol label: repeated colestipol doses before a propranolol dose reduced propranolol absorption. The reverse risk matters too — stopping colestipol without adjusting propranolol raises propranolol exposure.